

Virtual Tumor Board Report

Multi-Disciplinary Team Analysis

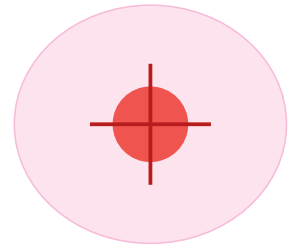
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Case Information

Cancer Site: Not specified

Clinical Stage: Not determined

Documents Analyzed: 0



Body

What Does This Stage Mean?

The cancer stage has not been fully determined yet. Additional tests like imaging or biopsies may be needed for accurate staging. Your oncologist will discuss this with you.

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TUMOR BOARD CONSENSUS

Good morning team. Thank you all for your insightful contributions to Anita Menon's case. We have a clear path forward for this 55-year-old woman with Stage IIIC High-Grade Serous Ovarian Carcinoma, notably with a BRCA1 germline mutation and HRD positivity.

Here is a synthesis of our discussions, leading to a consensus recommendation:

Each specialist provides their detailed assessment below:

Dr. Shalya - Surgical Oncology

Good morning team. Regarding Ms. Menon, 55-year-old with Stage IIIC HGSOE, BRCA1 germline mutation, and HRD positive.

My assessment on surgical options and timing is as follows:

1. Surgical Approach (PDS vs

Dr. Chikitsa - Medical Oncology

Dr. Chikitsa's Assessment:

This is a 55-year-old woman with newly diagnosed Stage IIIC HGSOE, notably with a BRCA1 germline mutation and HRD positivity, which are highly actionable biomarkers.

1. Optimal Surgical Approach (PDS

Dr. Kirann - Radiation Oncology

As a Radiation Oncologist, my assessment regarding the role of radiation therapy in this case is as follows:

For Stage IIIC High-Grade Serous Ovarian Carcinoma, radiation therapy (RT) plays a very limited to no role in the primary curative management.

1. Primary Treatment (Curative Intent):

Not Indicated: Radiation therapy is generally not a primary treatment modality for advanced epithelial ovarian cancer (EOC) like Stage IIIC HGSOE. The disease is typically widespread within the peritoneal cavity (T3c) and involves lymph nodes (N1), making it unsuitable for local or regional radiation with curative intent.

Historical Context: Whole Abdominal Radiation Therapy (WART) was explored historically but has largely been abandoned due to significant toxicity (e.g., renal, gastrointestinal, hepatic) and the advent of highly effective systemic chemotherapy.

Current Guidelines: NCCN and ESMO guidelines for advanced EOC focus on surgery (PDS or NACT-IDS) followed by platinum-based chemotherapy and appropriate maintenance therapy (PARP inhibitors or bevacizumab), especially in BRCA-mutated/HRD-positive cases like this. Radiation is not listed as a primary or adjuvant consolidative treatment.

2. Adjuvant/Consolidative Role:

There is no evidence to support the routine use of consolidative or adjuvant radiation therapy after optimal cytoreduction and systemic chemotherapy in this setting. The focus remains on systemic control and maintenance.

3. Potential Future Role (Palliative):

Radiation therapy may be considered in the recurrent setting for palliation of localized symptomatic disease. This could include:

Localized painful bone metastases.

Symptomatic brain metastases.

Localized pelvic or abdominal masses causing pain, bleeding, or obstruction, where systemic therapy alone is

Radiation Oncology (continued)

insufficient or not feasible.

In such palliative scenarios, modern techniques like Intensity-Modulated Radiation Therapy (IMRT), Volumetric Modulated Arc Therapy (VMAT), or Stereotactic Body Radiation Therapy (SBRT) would be employed to maximize dose to the target while minimizing exposure to surrounding healthy tissues.

In summary, for Anita Menon with Stage IIIC HGSOE, radiation therapy is not part of the initial treatment paradigm. The focus should remain on optimal surgical debulking, platinum-based chemotherapy, and subsequent maintenance therapy tailored to her BRCA1 mutation and HRD status.

References:

NCCN Clinical Practice Guidelines in Oncology: Ovarian Cancer, Version 1.2024.

ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up of epithelial ovarian cancer, 2023.

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Dr. Marga - Pathology

Dr. Marga, Pathology Specialist:

Thank you for presenting this case. The pathology and biomarker findings provide crucial information for guiding treatment and management.

Pathology and Biomarker Assessment:

1. Diagnosis Confirmation: The diagnosis of High-Grade Serous Ovarian Carcinoma

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Dr. Chitran - Onco-Radiology

Dr. Chitran (Onco-Radiology):

Thank you for presenting this comprehensive case.

Based on the Stage IIIC (T3cN1M0) diagnosis, it is presumed that a contrast-enhanced CT of the chest, abdomen, and pelvis has been performed, which

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Dr. Anuvamsha - Genetics

Good morning everyone. As the Genetics specialist, I will focus on the implications of Anita's genomic profile for her treatment and family.

Genomic Interpretation:

1. BRCA1 Germline Mutation (185delAG): This is the most significant finding.

Good morning team. Dr. Shanti here. Thank you for presenting Anita's case.

Given Anita is a 55-year-old with Stage IIIC HGSOE, BRCA1 germline mutation, and HRD positive, early and integrated palliative care is essential

IMPORTANT DISCLAIMER

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